POSTPROCEDURE

Introduction

In this lesson you will learn how to perform appropriate postcare for venipunctures and capillary punctures. These steps will include patient care such as bandaging the site and confirming that they are not experiencing any adverse effects from the blood collection. You will also learn how to label, handle, and transport the filled blood tubes to help provide the most accurate test results. Knowing the proper postcare for your patient is equally as important as performing the actual collection of blood. You will learn the best method of bandaging your patient to help reduce the risk of bruising and inspire healing. You will also learn to properly label the filled blood tubes or microtainers to help provide the most accurate test results. Lastly, you will learn how to observe and care for your patient throughout the postprocedure.

Postprocedure

Continue wearing the required PPE until all postcare steps and the venipuncture procedure have been completed. Even though the procedure is over, it is important to continue to observe your patient until they leave the area, because there is still a risk of a patient experiencing syncope, continuing to bleed, or feeling weak.

Apply pressure on the venipuncture site at least 5 to 10 seconds before bandaging or until the bleeding has stopped after a venipuncture procedure, lift the gauze from the site, and check to see if the bleeding has stopped. If the bleeding has not stopped, repeat the steps you learned about providing pressure after the collection. Once the bleeding has stopped, it is important to provide a pressure bandage to reduce the risk of infection and confirm that bleeding has stopped completely. If a bandage is not applied, slight bleeding may occur under the skin surrounding the needle insertion site, which will result in a bruise. If enough blood leaks from the area, it could result in a hematoma.

You will use an adhesive bandage, an adhesive wrap–style bandage, or tape and gauze to create a pressure bandage. Deciding which type to use is determined by what is available in your facility and patient-specific considerations. An older adult or young child may have tender, delicate skin, which may be torn by the adhesive of a typical bandage, making the adhesive wrap the better choice. On a person or location exposed to more movement than usual, the use of tape and gauze may provide a more secure and more flexible bandage.

4.7 CHALLENGE

1. If there is no sign of bleeding after venipuncture, do I have to bandage my patient?

4.7 CHALLENGE KEY

1. Even though it may appear that bleeding has completely stopped, you cannot see if any bleeding continues under the skin, so it is important to assist in the stoppage of blood with a pressure bandage. Bandaging also helps reduce the risk of infection.

TABLE 4.6 How to Apply Different Bandage Styles

BANDAGE STYLE	APPLICATION
Adhesive bandage	Unwrap and remove backing. Hold the bandage horizontally and apply one side of the bandage on one side of the site used for venipuncture. Confirming that the needle injection site is covered, pull taut and stick down on the other side.
Self-adhesive wrap	Tear off the size of the wrap comparable to the patient's arm or hand size. Place down over the folded gauze square and wrap around snugly, but not tightly enough to restrict blood flow. Firmly push the bandage down to self-adhere securely.
Tape and gauze	Tear off a piece of tape long enough to cover the site. Fold gauze into a square and place over the insertion site. Place one side of the tape down on outside of the site, over the gauze. Pull over gauze tightly and stick down the other end of the tape.

With all bandages, place on the skin snugly but not too tightly. When in doubt, ask your patient if it feels too tight and replace if necessary. Never use latex bandages on a patient with latex allergies. Avoid using too small of a bandage because it is difficult to provide adequate pressure on the site. Instruct the patient to leave the bandage in place for at least 15 min.

Labeling

After your patient has been properly bandaged, proceed to labeling the samples. Although tubes have a manufacturer's label, it is essential to label the tube to clearly indicate which patient the specimen was collected from. This indication is typically a preprinted label and is usually attached to the requisition.

Preprinted labels generally require:

- Patient's full name (or a unique code number in lieu of a patient name)
- Date of birth
- Unique patient identifier (medical record number, financial/account number, episode number)
- Date and time of collection
- Specimen collector's identification (ID; initials, signature, code) or space for this ID if it is to be manually written

4.8 CHALLENGE

1. A filled vacuum tube label should contain which of the following preprinted information?
   A. Name, date, provider's name, type of test
   B. Name, date of birth, room number, Social Security number
   C. Name, date of birth, date of admission, gender
   D. Name, date of birth, medical number, date and time of collection

4.8 CHALLENGE KEY

1. D. Name, date of birth, medical number, and date and time of collection should all be included on preprinted label. A patient may have several providers, and different tests could be processed on the same tube, so this information is not required to be placed on the specimen tube. A patient's room can change, and providing the Social Security number is a privacy violation. The date of admission is not relevant to a blood collection, and the sex or gender of the patient is not required on the label.